

Amblyopia reflects abnormal visual development

Reduced best-corrected acuity needs an organic explanation excluded before it is called amblyopia.

How the image becomes unequal or unavailable

Mechanism	Example	Treatment principle
Deprivation	Pupil-blocking cataract or ptosis	Remove the obstacle to a clear image.
Anisometropia	Different refractive power between eyes	Provide full optical correction.
Strabismic suppression	One eye consistently favoured for fixation	Encourage use of the weaker eye after assessment.
Crowding	Isolated letters look better than a full line	Use suitable crowded acuity testing.

Treatment uses the weaker eye

Prescribe the optical correction and address any visual-axis obstruction. When indicated, patch the better-seeing eye so the amblyopic eye takes up fixation; atropine penalisation is another prescribed approach. Family understanding and follow-up matter because treatment targets a developing visual system. Do not diagnose amblyopia from a low chart score without examining the eye.

Cover testing reveals alignment

Watch the uncovered eye for a manifest deviation; alternate cover reveals a latent one.

Movement and interpretation

Observation	Interpretation
Cover right; uncovered left moves outward	The left eye was inward: left esotropia.
Cover right; uncovered left moves inward	The left eye was outward: left exotropia.
No manifest movement; outward drift under alternate cover	A latent outward deviation is revealed.
Eye moves inward after alternate uncovering	It refixates from an exophoric position.

Avoid confusing the tests

The cover test asks whether an uncovered eye must move to take up fixation, identifying a tropia. Alternating the cover dissociates binocular fusion and can reveal a phoria. Full movements and a similar angle in different gaze positions suggest a comitant pattern; limited movement or a changing angle requires assessment for an incomitant cause.